

Standard Operating Procedure:

HR.S.09 Annual leave Guidance for all Medical & Dental Staff

Please also see HR.78 for policy detail

Document Summary

The aim of this guidance is to provide a consistent, efficient and equitable approach to the calculation, management and application of leave for all Medical and Dental staff employed at the Trust, allowing due choice in the selection of leave, together with the effective utilisation of resources, ensuring clinical safety at all times.

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1 SCOPE AND PURPOSE

The aim of this guidance is to provide a consistent, efficient and equitable approach to the calculation, management and application of leave for all Medical and Dental staff employed at the Trust, allowing due choice in the selection of leave, together with the effective utilisation of resources, ensuring clinical safety at all times.

The Trust recognises that annual leave is an important part of a practitioners work-life balance and is committed to ensuring they are able to take their contractual annual leave in full during the relevant leave period. Good management of annual leave by the practitioner and the manager is essential to the health and safety of the practitioner and patients.

This guidance document is to be used in conjunction with the Annual Leave Policy for all Medical and Dental staff (HR.78).

2 PROCEDURE

All new practitioners, on joining the trust, will be given an entitlement to annual leave plus bank holidays as set out in Trust policy. This is on a pro-rata basis for less than full time practitioners and will run from the first date of employment forward unless an alternative common start date has been agreed.

Annual Leave Entitlements – are based on reckonable service and hours of work up to a maximum full time week (40hrs/10PA's) and once calculated will be shared with the rota team and the General Manager/Clinical Lead. The practitioner is responsible for checking their entitlement in conjunction with these individuals and any queries should be raised as soon as possible.

Annual Leave Planning - cannot be emphasised enough, so it is crucial to plan leave carefully to ensure practitioners have regular rest. These roles can be very stressful and it is important to maintain health and well-being, ensuring annual leave is planned well in advance. It is recommended it is spread across the leave year. An example of a uniform spread is given below but this may be varied across the leave year according to both operational and individual circumstances.

25% quarterly leave target with no carry over	Days	Quarter	Days	%
Days leave	36	Q1	11	25%
Public holidays	8	Q2	11	25%
Annual leave total	44	Q3	11	25%
		Q4	11	25%
		Total	44	100%

All annual leave should endeavour to be taken within the leave year. Any annual leave not taken within the relevant leave year may not be authorised for carry over into a subsequent year unless in extenuating circumstances.

The General Manager and or Clinical Lead for your work area, will analyse leave requests made against operational demands in the given year to inform any request for carry over submitted. Where authorised, up to 5 days annual leave may be carried over this will be pro-rata for part-time staff. The application form can be found as **Appendix 3** of the Annual Leave Policy for all Medical and Dental Staff (HR.78).

Please note, the practitioner would be expected to use any carryover of leave in the first three months of the subsequent leave year, in addition to the 25% of leave entitlement to be taken. If this doesn't happen the approval of carry over may be declined. An examples of how this might look are given below.

25% quarterly leave target with carryover Q1	Days	Quarter	Days	%
Days leave	36	Q1	15	31%
Public holidays	8	Q2	11	23%
Annual Leave total	44	Q3	11	23%
Carry over	4	Q4	11	23%
Annual leave + carryover total	48	Total	48	100%

The proportional effect of taking leave – must equate to no more than a practitioners full annual entitlement to leave, i.e. if a practitioner chooses to take their leave in periods of time shorter than one week, the loss of clinical activity over the year must not equate to more than the annual leave entitlement of their normal average weekly timetable as defined by their job plan.

When booking leave, it should reflect the proportion of direct clinical care (DCC), supporting professional activities (SPA) or other commitments within the practitioner's job plan. Particular care should be taken when leave is taken in single days to ensure that there is not a disproportionate effect on any one activity e.g. infrequent clinics, theatre lists or meetings. This will include taking leave for flexibly delivered activities that would routinely be delivered within a normal working week, not simply what is visible on the timetable of the job plan or rota.

For example, a consultant who has a job plan that includes 6 clinical sessions per week, would normally expect that their total annual leave for the year would result in the loss of no more than 36 clinical sessions (plus equivalent allowance for any applicable bank holidays not worked).

Complicated rotas and on call arrangements – exist within some specialties so all areas are required to build and update regularly, local departmental rota rules. A template to support the build of these can be found in the Trust's Guidance on Annual Leave Management in Rota's.

The rules will be reviewed by the management team regularly within area and agreed with the ACO and C/D by the G/M and C/L before being discussed in each specialty at the team job planning meeting at least annually.

Normal working week - for the purposes of annual leave entitlement, consists of whatever constitutes the member of staff's average normal working week e.g. if a consultant works a normal working week of 5 days, this is the number of days required to consume a weeks' worth of leave.

If a full time job plan consists of a fixed day off and a 4 day working week, leave will need to be averaged over these 4 days to ensure the relevant number of days taken consume a weeks' worth of leave provided in entitlement i.e. 1.25 days x 4 and so on.

Normal working day - If a consultant works a normal working week of 5 days, each working day may be split as 1/5th if delivery is equally spread or where long days consume more contracted PAs then these must also consume more of the planned leave entitlement.

If the job plan consists of a fixed day off and a 4 day working week, this is the number of days required to consume a weeks' worth of leave. In this example each working week day may be split

as 1/4th if delivery is equally spread or where long days consume more contracted PAs these must also consume more of the planned leave entitlement. As explained above.

Normal working day – Public Holidays - For the calculation of a public holiday, 8 hours/2 PA's/1 day is used as standard for a normal working day calculation, this is calculated as 1/5th of the standard working week (40 hours over 5 days) and pro rata'd for part time staff.

Where a practitioner has been scheduled to work on a bank holiday - they will retain a day's leave. For this purpose, work is defined as necessary attendance at the hospital for clinical reasons, and/or a formal on call commitment of 8 hours or more where the practitioner is called in to attend the hospital or receives multiple/significant disruptions with phone calls.

Examples are given below:

Public/General Holidays – worked on site	Time Retained
Where required to work one shift e.g. 8am until 5pm	1 day retained
Where one shift spans 2 public e.g. 9pm until 9 am	2 days retained
Where two public holidays fall together i.e. Christmas day/boxing day and where one shift is worked on each of these days e.g. 8am until 5pm Christmas day and 8am until 5pm boxing day	2 days retained
If the public holiday falls on a weekend and re-designated to the Monday or Monday/Tuesday (this only happens with Christmas day, boxing day and New Year's day).	Lieu days will be given for working either Christmas day, Boxing day or New Year's day or the re-designated Public holiday but not both. Maximum 2 lieu days given in total.
Public/General Holidays – Formal on call commitment	Time in Lieu given
On call from home midnight to 9am on the public holiday but called in to attend the hospital or receives multiple/significant disruptions.	1 lieu day given
On call from home midnight to 9am on the public holiday but not called in to attend the hospital and receives no disruption	1 lieu day given
Daytime (8 hours or more commitment) and called in to attend the hospital	1 lieu day given
Daytime (8 hours or more commitment) not called in to attend the hospital	1 lieu day given

Any changes to leave entitlements – When an employee's leave entitlement changes during the leave year because of an increase in length of service or a change in hours, the new entitlement should be calculated from the date of the service anniversary, or the date of hours change.

Cover arrangements for prospective work i.e. weekends/nights/on call - are the responsibility of the practitioner to swap. No leave will need to be applied for nor granted where these shifts fall. If the practitioner is having difficulty in identifying a suitable alternative colleague to cover these shifts, support can be sought from the relevant rota team. Responsibilities also cover the handing over of seriously ill patients or those who have undergone surgery whose changing condition may require clinical intervention.

All General Managers/Clinical Lead and responsible rota co-ordinators, will review rosters at six, four, two and one week intervals ahead of the roster being implemented and in accordance with the Trust's Guidance on Annual leave management in rotas. This is to ensure that there is coverage and any requirements for locums are known and booked well in advance.

Time in Lieu (TOIL) - where clinicians have done additional work and elected to have time off in lieu rather than payment for this work, this needs to be managed by the General Manager / Clinical Lead and the time earned recorded as per the Trust's TOIL guidance. This is not to be recorded as part of leave entitlements in trust electronic systems. It is advised that lieu time is booked before using annual leave allocation.

Failure to return from annual leave on an agreed date - may occur in exceptional circumstances. In these instances the practitioner is expected to contact their General Manager / Clinical Lead as soon as possible to discuss the situation. Depending on the circumstances (flight delay, natural disaster, illness etc.) and the expected length of additional absence, they may be required to take additional leave from their paid holiday entitlement, unpaid leave or time in lieu.

Where a practitioner does not return from leave on the date agreed and has not made contact with their General Manager / Clinical Lead to discuss their circumstances, the absence will be treated as unauthorised and further formal action may be deemed necessary.

Popular Periods of Leave - e.g. school holidays, Christmas and Bank Holiday weeks will affect leave requests for some staff so fair treatment and the opportunity to have these popular periods of time off will be considered equally for all.

Leave will need to be calculated/ booked - in either days or hours within electronic systems or depending on the contract and as referred to in the Trust's policy.

The working year is taken as 42 working weeks to account for annual and study leave, statutory days and bank holidays. It will be slightly less for those who have additional leave related to their date of appointment/length of service. However if study leave is not requested and approved within the leave period this working year will increase as it cannot be substituted for additional annual leave.

Where a doctor works five days and they take one week's leave they must book 5 days leave. Where it has been agreed that a doctor works a condensed week. i.e. 4 long days and has the fifth day as a non-working day. The annual leave for the year would be calculated on the basis of a 4 long days in a working week i.e. 1.25 days per day booked.

An electronic calculator for establishing the average working week and which will support accurate leave deductions is provided within the Standard operating Procedure for Rota management but some examples of how leave should be deducted depending on the average working week is given below.

Example 1

A practitioner has a full time/10 PA or more job plan and there is no on-call associated with the post but hours are worked over 4 days

The average working week would mean 2.5 PA's worked a day or $4/5 = 10$ hours/1.25 days to be deducted per day of leave taken.

Example 2

A practitioner has a 10 PA job plan and has worked as a consultant for 7 years, the job plan indicates 3 long days of work each week.

The average working week would mean 3.33 PA's worked a day or $3/5 = 13.33$ hours/1.67 days.

Example 3

A practitioner has an 8 PA job plan and has worked as a consultant for 7 years, the job plan indicates 4 long days of work each week.

The average working week would mean 2 PA's worked a day = 8 hours/1 day.

Example 4

Where a practitioner works five days on a 10 PA job plan and has one day that has no fixed clinical commitments CPD / SPA / Flexible activity but they wish to take annual leave during that day and will not be available for work, they are required to book annual leave.

All non DCC activity must be booked when taking leave.

The average working week would mean 2 PA's worked a day or $1/5 = 8$ hours/1 day to be deducted per day of leave taken.

Further to this, if a practitioner wishes to go on holiday from Monday to Wednesday and is therefore not available for work on the Wednesday, they are required to book three days leave which includes the Wednesday.

It is important to note that should they not book leave on the Wednesday, and it is later proven that they have failed to book the appropriate amount of leave, this could be constituted as fraud and an investigation undertaken.

Similarly, a practitioner with a 12 PA job plan, who works four days one week and 5 days the next and is going on holiday for two weeks, on the weeks when the practitioner is planning to take leave, they have no fixed clinical commitments on the Monday of the first week or the Friday of their second week. They travel on Monday of week 1 and do not return until the Sunday of week 2.

They are required to book 10 days/20 PA's/80 hours annual leave as they are not available to work during that time, as annual leave must be booked for both clinical and non-clinical days.

Entitlement on leaving the Trust - will be calculated through electronic systems.

3 REFERENCES

Annual leave Policy for all Medical & Dental Staff (HR.78)

Standard Operating Procedure – Rota Management for All Medical & Dental Staff

National terms and conditions for Consultants, SAS and Resident doctors

NHS Employers website

BMA Website

ACAS